

VETERINARY MEDICAL REPORT

CLINIC INFORMATION

Facility: Sunrise Animal Hospital
Address: 456 Oak Street, Springfield
Phone: (555) 123-4567
Date of Examination: November 2, 2025
Time: 14:30

PATIENT INFORMATION

Patient Name: Max
Species: Canine
Breed: Labrador Retriever
Age: 4 years
Sex: Male (Neutered)
Weight: 32 kg (70.5 lbs)
Color/Markings: Yellow
Microchip #: 985112001234567

Owner Name: Sarah Johnson
Owner Contact: (555) 987-6543
Owner Address: 123 Maple Drive, Springfield

CHIEF COMPLAINT

Active bleeding from left hind leg following outdoor activity. Owner reports dog was running in wooded area and yelped approximately 45 minutes prior to presentation.

HISTORY OF PRESENT ILLNESS

Max was off-leash during morning walk in local park when owner heard him yelp. Upon inspection, owner noticed active bleeding from the left hind leg, approximately mid-way between the hock and stifle. Owner applied pressure with clean cloth and brought dog directly to clinic. Dog is current on all vaccinations including rabies (last administered March 15, 2025). No known drug allergies. No prior surgical history.

VITAL SIGNS

- **Temperature:** 38.7°C (101.7°F) - Normal
 - **Heart Rate:** 96 bpm - Normal
 - **Respiratory Rate:** 28 breaths/min - Slightly elevated
 - **Mucous Membranes:** Pink, moist, CRT < 2 seconds
 - **Pulse Quality:** Strong and regular
 - **Body Condition Score:** 5/9 (Ideal)
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PHYSICAL EXAMINATION

General Appearance: Alert, responsive, mild anxiety noted. Ambulatory but favoring left hind limb.

HEENT: Eyes clear, no discharge. Ears clean. No nasal discharge. Mucous membranes pink and moist.

Cardiovascular: Heart sounds normal, no murmurs detected. Strong femoral pulses bilaterally.

Respiratory: Lungs clear bilaterally, no abnormal sounds.

Abdomen: Soft, non-painful on palpation. No organomegaly detected.

Musculoskeletal: Normal gait except for left hind limb lameness (grade 2/4).

Neurological: Conscious, alert, responsive. Normal cranial nerve function.

Integumentary/Wound Assessment:

- **Location:** Left hind leg, lateral aspect, mid-tibia region
 - **Wound Type:** Laceration, approximately 4 cm in length
 - **Depth:** Full-thickness, extending through dermis into subcutaneous tissue
 - **Appearance:** Irregular edges suggesting trauma from sharp object (likely branch or debris)
 - **Bleeding:** Moderate active bleeding controlled with direct pressure
 - **Contamination:** Moderate debris present (dirt, plant material)
 - **Surrounding Tissue:** Mild edema, no crepitus, no obvious fracture on palpation
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DIAGNOSTIC PROCEDURES

Wound Cleaning and Exploration:

- Area clipped and surgically prepared
- Wound irrigated with sterile saline solution
- Foreign material (small wood fragments) removed
- No evidence of joint penetration
- No tendon or ligament damage visualized

Radiographs (Left Tibia/Fibula - 2 views):

- No fracture identified
 - No radiopaque foreign bodies detected
 - Soft tissue swelling consistent with trauma
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ASSESSMENT/DIAGNOSIS

1. **Traumatic laceration, left hind limb** - full-thickness wound with moderate contamination
 2. **Mild soft tissue trauma** - surrounding edema without underlying fracture
 3. **Risk for wound infection** - contaminated wound requiring wound management
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TREATMENT PLAN

Immediate Treatment Administered:

1. Pain Management:

- Carprofen 2.2 mg/kg PO (70 mg administered)

2. Sedation for Wound Repair:

- Dexmedetomidine 5 mcg/kg IM
- Butorphanol 0.2 mg/kg IM

3. Wound Management:

- Copious irrigation with sterile saline
- Debridement of devitalized tissue
- Primary closure with 3-0 Monocryl (subcutaneous) and 3-0 Nylon (skin)
- Total of 8 interrupted sutures placed
- Sterile non-adherent dressing applied
- Soft padded bandage with elastic wrap

4. Antimicrobial Therapy:

- Cephalexin 22 mg/kg PO BID (700 mg per dose) - started today

5. Tetanus Prophylaxis:

- Tetanus antitoxin 500 IU administered SC
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DISCHARGE INSTRUCTIONS

Medications:

1. Cephalexin 700 mg: Give 1 capsule by mouth twice daily for 10 days
2. Carprofen 75 mg: Give 1 tablet by mouth twice daily for 5 days (with food)

Wound Care:

- Keep bandage clean and dry
- Return in 3 days for bandage change and wound check
- Prevent licking/chewing with E-collar (provided)
- Monitor for signs of infection: increased swelling, discharge, odor, heat

Activity Restriction:

- Strict rest for 10-14 days
- Leash walks only for elimination
- No running, jumping, or rough play
- No swimming or bathing until sutures removed

Follow-Up:

- **Recheck appointment:** November 5, 2025 (3 days) for bandage change
- **Suture removal:** November 16, 2025 (14 days)

Emergency Instructions: Contact emergency veterinarian immediately if:

- Excessive bleeding through bandage
- Bandage becomes wet or soiled
- Signs of pain not controlled by medication
- Lethargy or loss of appetite
- Swelling above or below bandage
- Foul odor from wound

PROGNOSIS

Good. With appropriate wound care and antimicrobial therapy, wound is expected to heal without complications. Full return to normal activity anticipated within 2-3 weeks.

CHARGES SUMMARY

- Office Examination: \$85.00
- Radiographs (2 views): \$145.00
- Sedation & Monitoring: \$120.00
- Wound Repair/Suturing: \$285.00
- Medications (10-day supply): \$65.00
- Bandaging materials: \$35.00
- E-collar: \$25.00
- **Total:** \$760.00

Payment Status: Paid in full

Attending Veterinarian: Dr. Michael Rodriguez, DVM

License #: VT-45892

Signature: *Dr. M. Rodriguez*

Date: November 2, 2025

This report is confidential and intended solely for the pet owner and referring veterinary professionals.